INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Zoledronic acid · adjuvant breast treatment

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment timing

Your plan

See the schedule in your treatment guide

Regimen source

NCCP 00545

Version 3

> An infusion every six months

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Reduce recurrence risk as part of the agreed early-breast treatment plan.

BONE-MODIFYING BISPHOSPHONATE

Zoledronic acid

Slows bone breakdown by osteoclasts

Binds to bone → Osteoclast activity reduced → Bone turnover slows

Zoledronic acid binds to bone and reduces the activity of cells that break bone down. In selected people with early breast cancer it forms part of the adjuvant plan. It does not replace surgery, radiotherapy, chemotherapy or endocrine treatment.

Report jaw pain or a wound that will not heal, reduced urine, muscle cramps, eye pain or new thigh/groin pain.

Why this treatment?

This may be added to an adjuvant plan after discussing recurrence risk, menopause status, kidney function, dental health and the expected benefit. It is a separate treatment from medicines used to control bone metastases.

Your treatment setting

Adjuvant breast cancer · postmenopausal (natural or treatment-induced)

NCCP 00545 v3 · official regimen

Your exact schedule

Zoledronic acid by intravenous (IV) infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

NCCP 00545 v3 · official regimen

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

NCCP 00545 v3 · official regimen

Why this treatment may be chosen

Discuss your own expected benefit, alternatives and uncertainty with the treating team.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report jaw pain, exposed bone or a non-healing mouth wound, reduced urine, tingling or muscle spasms, painful red eyes, or new thigh/groin pain.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00545 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Zoledronic acid · adjuvant breast treatment: treatment and checks

Your treatment pattern

An infusion every six months

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00545	Endocrine-depleted setting	First infusion ≥15 minutes	Every 6 calendar months	Up to 5 years; review continuation	Bone / cancer follow-up
Kidney / calcium NCCP 00545	Creatinine, creatinine clearance (CrCl), calcium, Mg / phosphate	Confirm suitable results	Creatinine before every infusion	Review deterioration / recovery	Individual bone/renal plan
Dental / supplements NCCP 00545	Dental assessment / prevention	Prescribed calcium and vitamin D	Dental care and symptom review	Plan invasive procedures	Report jaw / thigh symptoms
Cancer review EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1 **Before treatment**

Before the first dose

Adjuvant breast cancer · postmenopausal (natural or treatment-induced). Before starting: renal profile, magnesium, calcium and phosphate. Check creatinine before every infusion; compare it with baseline as well as reviewing creatinine clearance. Correct low calcium and review vitamin D.

Ask / **act:** Confirm eligibility, test results and medicine list.

2 **Other checks**

At the specified review points

Complete a dental assessment and preventive care before treatment where appropriate. Report dental symptoms; discuss invasive dental work with oncology and dental teams. Hydration is individualised, especially with heart failure.

Ask / **act:** Confirm who arranges and reviews each check.

3 **Support and practical plan**

At each treatment or phase change

Review renal function, calcium, vitamin D, dental health and hydration before each infusion.

Ask / **act:** Choose the matching support plan; confirm active medicines.

4 **Response and follow-up**

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / **act:** Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

Before starting: renal profile, magnesium, calcium and phosphate. Check creatinine before every infusion; compare it with baseline as well as reviewing creatinine clearance. Correct low calcium and review vitamin D.

NCCP 00545 v3 · official regimen

Other treatment-specific checks

Complete a dental assessment and preventive care before treatment where appropriate. Report dental symptoms; discuss invasive dental work with oncology and dental teams. Hydration is individualised, especially with heart failure.

NCCP 00545 v3 · official regimen

Cancer review and follow-up

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

NICE NG101 · bone health and follow-up

The wider treatment plan

Agree surgery, radiotherapy and endocrine treatment as applicable; their schedules and checks remain separate.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

NCCP 00545 v3 · official regimen

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

NCCP 00545 v3 · official regimen

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

NCCP 00545 v3 · official regimen

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Six-monthly treatment and safety reviews

No dates are entered. Columns show milestones, not equal time intervals; follow your individual prescription.

Treatment or check	Timing in this view
Before starting	**Before treatment** Eligibility, tests and medicines review
Treatment	**First infusion → Six-monthly reviews → Later infusions** One infusion at each agreed six-monthly visit
Monitoring / support	**First infusion → Six-monthly reviews → Later infusions → Completion** Keep the treatment-specific test and symptom plan
Next decision	**Completion** Confirm continuation or completion

The interval is six months, not a 180-day chemotherapy cycle.

The source permits up to five years; your team agrees the duration.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Calcium and vitamin D

Follow the prescribed daily supplements; the team checks calcium and adjusts them if it is high or low.

Dental care

Keep routine dental care and tell the dentist about treatment. Report jaw pain or a wound that does not heal.

Flu-like symptoms

Aches or fever can occur after infusion. Use the agreed symptom plan; seek advice if severe or persistent.

Hydration

Follow the team's fluid advice; heart or kidney problems may change it.

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

Preparation, blood review and observation make the visit longer.

A port or take-home infusion pump is not routinely required for this medicine alone.

In the unit

A peripheral cannula is often sufficient.

Questions to ask

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Explain the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Before each six-monthly infusion

Active medicines: Zoledronic acid

Emetogenic risk: Minimal

Medicine	Dose / route	When
Calcium	At least 500 mg orally daily	Daily supplementation unless hypercalcaemia is present Review diet, calcium results and the prescribed preparation.
Vitamin D	At least 400 IU orally daily	Daily supplementation unless hypercalcaemia is present Review deficiency, calcium results and individual supplementation needs.

Additional phase-specific support

- No routine antiemetic premedication is required.
- Ensure adequate hydration; avoid overhydration in people at risk of heart failure.
- Review dental health and symptoms, calcium status, renal results and any nephrotoxic medicines.
- Hold/restart and renal dose decisions need the current NCCP assessment and prescriber review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

Zoledronic acid · adjuvant breast treatment

Zoledronic acid by intravenous (IV) infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

Adjuvant bone-modifying treatment · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Jaw / mouth

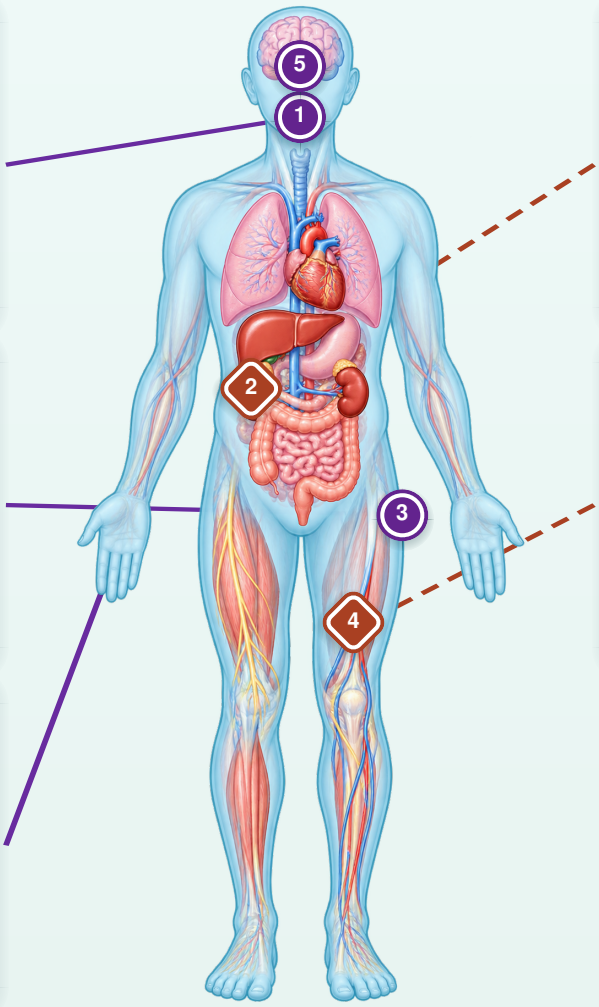
Zoledronic acid

Report jaw pain, exposed bone or a wound that does not heal.

2 Kidneys / urine

Zoledronic acid

Report reduced urine or marked swelling.



3 Muscles / calcium

Zoledronic acid

Tingling, cramps or spasms may need urgent blood tests.

4 Thigh / groin

Zoledronic acid

Report new unexplained thigh or groin pain.

5 Eyes

Zoledronic acid

Painful red eyes or vision changes need prompt review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report jaw pain, exposed bone or a non-healing mouth wound, reduced urine, tingling or muscle spasms, painful red eyes, or new thigh/groin pain. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Jaw / mouth symptoms	☐	☐	☐	
Muscle / joint pain	☐	☐	☐	
Urinary changes	☐	☐	☐	
Eye symptoms	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Report jaw pain, exposed bone or a non-healing mouth wound, reduced urine, tingling or muscle spasms, painful red eyes, or new thigh/groin pain. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Zoledronic acid · adjuvant breast treatment

NCCP 00545 v3

Zoledronic acid by IV infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Report jaw pain, exposed bone or a non-healing mouth wound, reduced urine, tingling or muscle spasms, painful red eyes, or new thigh/groin pain.

https://sactcheck.com/patient/00545/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00545 v3 · official regimen**
   https://healthservice.hse.ie/documents/6381/545_Zoledronic_acid.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
5. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
7. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
8. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
11. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext